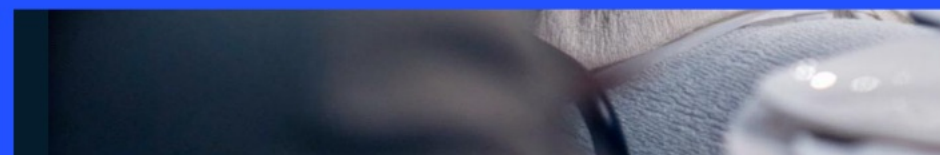


McKinsey Case Championship - TechQuest

Case Title: Global Circuits, Local Challenges – SummitBridge Health Plan
Medical cost Management challenge

May 2025

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Case Context

You are a newly hired Healthcare Analytics Consultant at SummitBridge Health Plan, a US based mid-sized regional health insurance company operating in 6 states across the Midwest and Southeast.

Despite strong clinical partnerships and competitive premiums, SummitBridge is under pressure on medical cost management. Leadership has flagged two major cost drivers:

High-cost utilization - Specialty drugs, inpatient stays, and emergency department (ED) visits that may be avoidable or better managed in lower-cost settings

Inefficient care patterns - Duplicative services, out-of-network leakage, and gaps in preventive/primary care that drive downstream cost

Organizational Details

•**Products:** Commercial employer-sponsored plans, Individual Marketplace (ACA), Medicare Advantage

•**Scale:** ~420,000 covered lives across 3 product lines

•**Network:** ~18,000 contracted providers (PCPs, specialists, hospitals, pharmacies)

•**Annual medical spend:** ~\$2.1B in allowed claims

Key Issues Identified

Rising High-Cost Claimants - A small share of members drives a disproportionate share of spend (specialty pharmacy, oncology, behavioral health)

Avoidable & Potentially Avoidable ED Utilization - ED visits that could have been treated in urgent care or primary care

Out-of-Network (OON) Leakage - Members receiving care outside the contracted network, driving higher allowed amounts and balance billing risk

Member Retention & Satisfaction - Commercial group renewals are slipping; exit surveys cite cost confusion and access issues

Goal: The goal is to develop a strategy to optimize the utilization of premium and non-productive hours, reduce labor costs, and improve retention, while maintaining high-quality patient care; along with creating a dashboard to track performance on a regular basis

Tasks

1. High-Cost Claimant & Specialty Spend Optimization

- **Analyze:** Examine claims patterns for the top cost drivers (specialty drugs, inpatient, ED, behavioral health). Identify concentration (e.g., top 5% of members by PMPM) and trends by product line and region.
- **Propose:** Recommend interventions with evidence and action (e.g., prior authorization refinement, site-of-care shifts, care management outreach, formulary adjustments).
- **Calculate:** Model savings if specialty pharmacy allowed is reduced by 5%, 10%, and 15%, and if 20% of avoidable ED visits shift to urgent care (use average allowed per visit for each setting).

2. Avoidable Utilization Reduction

- **Identify:** Pinpoint main contributors to potentially avoidable ED visits, Behavioral health and readmissions,. Segment by age, product, geography, and PCP attribution. Behavioral health spend is low because of volume or unit cost? Do members with BH claims have different ED or inpatient patterns?
- **Recommend:** Propose solutions (telehealth routing, PCP access improvements, ED diversion programs, discharge planning).
- **Develop:** Outline a plan for ongoing monitoring and develop KPIs (e.g., PCP visit rate within 7 days of ED visits, 30-day readmission rate).

3. Member Experience & Retention

- **Suggest:** Propose initiatives to improve satisfaction and reduce churn (clearer cost transparency, network adequacy in rural counties, digital tools for finding in-network care).
- **Analyze:** (1) Use enrollment data to identify cohorts with highest disenrollment and correlate with access gaps or cost exposure. (2) Do members attributed to lower quality score PCPs have higher PMPM? Is this confounded by geography or LOB?

4. Data-Driven Decision-Making

- **Design:** Define a framework for integrating claims, provider, and enrollment data—including KPIs, refresh cadence, and ownership.
- **Build :** Create a simple dashboard or notebook that tracks PMPM allowed, IP utilization, ED utilization, OON rate, and high-cost member concentration.
- **Explain:** Articulate how analytics can improve utilization management and support actuarial/pricing decisions

Data shared, Assumptions and Guidelines

Data Provided		Assumptions and Guidelines
File	Purpose	
`claims.csv`	Medical and pharmacy claim lines	- Use allowed amount as the primary cost metric unless noted otherwise.
`enrollment.csv`	Member demographics, enrollment, and churn	- Analysis period: January 1, 2024 – December 31, 2024
`providers.csv`	Provider network attributes	- A member is high-cost if they are in the top 5% of allowed spend in the period.
`avoidable_ed_reference.csv`	ICD-10 lookup for ED classification	- PMPM = Total Allowed Amount ÷ Member Months.
`member_months.csv`	Pre-calculated member-month exposure	- ED visits per 1,000 = (ED claims ÷ member months) × 1,000.
		- Document all joins, filters, and assumptions in your workbook..